

Presmol 500mg tablets

DESCRIPTION

Round, 12 mm in diameter, orange film-coated tablet, bevel edged, shallow convex faces; "SETAMOL" embossed on one face.

COMPOSITION

Paracetamol 500 mg/tablet.

PHARMACODYNAMICS

Paracetamol has analgesic and antipyretic actions but no useful anti-inflammatory properties. It may act by inhibiting prostaglandin synthesis in the central nervous system (CNS) and through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of the synthesis of prostaglandins or to inhibition of the synthesis or actions of other substances, which sensitize pain receptors to mechanical or chemical stimulation.

Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

PHARMACOKINETICS

Absorption

Rapid and almost complete following oral administration; may be decreased if taken following a high-carbohydrates meal.

Blood Concentration

After an oral dose of 1g to fasting subjects, peak plasma concentration of 20 to 30 µg/ml are attained in about 20 minutes. After giving the same dose to non-fasting subjects, peak plasma concentrations of 10 to 20 µg/ml are attained in 40 to 120 minutes.

After an oral dose of 2g to fasting subjects, peak plasma concentrations of about 30 to 40 µg/ml are attained in 1 to 3 hours.

Half-life

Plasma half-life after therapeutic doses is approximately 1 to 4 hours; does not change with renal failure but may be prolonged in some forms of hepatic disease, in the elderly, and in the neonate, and may be somewhat shortened in children.

Distribution

Widely distributed throughout most body fluids; secreted in the saliva in concentrations paralleling those in the plasma; volume of distribution, about 1 litre/kg body weight.

Protein Binding

Plasma-protein binding is negligible with doses producing plasma concentrations below 60 mcg per ml but increase with increasing concentrations.

Metabolic Reactions

Approximately 90 to 95% of a dose is metabolized in the liver, primarily by conjugation with glucuronic acid, sulfuric acid, and cysteine. The metabolite formed by oxidation, which is hepatotoxic, is conjugated by the glutathione pathway to form mercapturic acid and cysteine conjugates.

Excretion

About 85% of a therapeutic dose is excreted in the urine in 24 hours; of the excreted material in adult, 1 to 4% is unchanged, 20 to 30% is conjugated with sulphate, 40 to 60% is conjugated with glucuronic acid, 5 to 10% is composed of 3-hydroxy-3-sulphate, 3-methoxyglucuronide, and 3-methoxy-3-sulphate metabolites, and 5 to 10% is composed of mercapturic acid and cysteine conjugates. Larger amounts of the mercapturic acid and cysteine conjugates are excreted in overdose.

INDICATIONS

For temporary relief of occasional minor aches, pains and headache and reduction of fever.

CONTRAINDICATION

Patients hypersensitive to aspirin may not be hypersensitive to paracetamol; however, mild bronchoplastic reactions with paracetamol have been reported in some aspirin-sensitive asthmatics (less than 5% of those tested).

WARNINGS AND PRECAUTIONS

- Caution in patients with hepatic or renal function impairment.
- Check with physician if symptoms do not improve or if fever lasts longer than 3 days or recurs.
- Chronic use should be avoided, unless otherwise directed by physician.
- Serious skin reactions:
Rarely, paracetamol may cause serious skin reactions such as acute generalised exanthematous pustulosis (AGEP), Steven-Johnson

syndrome (SJS), and toxic epidermal necrolysis (TEN), which can be fatal. Patients should be informed about the signs of serious skin reactions, and use of the drug should be discontinued at the first appearance of skin rash or any other sign of hypersensitivity.

- Allergy alert:
Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash.
These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

MAIN SIDE/ADVERSE EFFECTS

In recommended therapeutic dosage, paracetamol is usually well tolerated. Rarely, hypersensitivity and haematological reactions may occur. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

PREGNANCY AND LACTATION

None known.

DRUG INTERACTIONS

Risk of hepatotoxicity with single toxic doses or prolonged use of paracetamol may be increased in chronic alcoholics or in patients regularly taking other hepatic enzyme-inducing agents.

OVERDOSE

Clinical features: Nausea, vomiting, epigastric discomfort, hypoprothrombinaemia, hepatic or renal failure, metabolic acidosis and coma in very severe cases.

Treat overdosage by emesis or gastric lavage, if indicated. Administer N-acetylcysteine orally (start within 10 hours after ingestion), 140 mg of 20% solution per kg as a loading dose, followed by 70 mg/kg every 4 hours for 3 days with the necessary supportive treatment if required.

DOSAGE AND ADMINISTRATION

For oral use.

Adult : Oral, 2 tablets every 4 hours, not exceeding 8 tablets daily.
Children (7 to 12 years) : Oral, 1 tablet every 4 hours, not exceeding 4 tablets daily.

The information given here is limited. For further information consult your doctor or pharmacist.

Storage : Store below 30°C. Protect from moisture.
Presentation/Packing : Film-coated tablet 500 mg;
Blister of 10 x 10's, Blisters of 100 x 10's.

Product Registration Holder: Hovid Pharmacy Sdn. Bhd.,
121, Jalan Tunku Abdul Rahman, 30010, Ipoh, Perak.

Manufactured by: Hovid Bhd.,
Lot 56442, 7 1/2 Miles Jalan Ipoh/Chemor,
31200, Chemor, Perak, Malaysia.

Distributor: Sunrise Pharma Sdn Bhd
Unit 8, Lot 20 (Type A), Industrial E33, Likas, Mike 2 1/2,
Jalan Tuaran, Likas, 88000 Kota Kinabalu, Sabah.

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